

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: ThornburgPeriod: April 1 to April 30, 2026Report Date: May 1, 2026Coverage: 293/720h (41%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Last 24 Hours

Elevated Breathing

12 AM to 1 AM

Vital Stat	Avg	Min	Max
Heart Rate	61 bpm	52 bpm	73 bpm
Breathing	22 brpm	14 brpm	32 brpm

Last 24 hours: two elevated breathing episodes overnight, otherwise within normal range.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
12 AM to 1 AM	2h	2	Elevated Breathing	24	20	32	Check SpO2 and lung sounds; assess for fluid overload or early infection

Patient clinical status over 30 days from Apr 01 to Apr 30

HR Breathing

Elevated Breathing

Apr 25 to Apr 25 (1d)

Elevated Breathing

Apr 28 to Apr 30 (3d)

10 episodic events Detected, spanning 27h total. Conditions: Elevated Breathing. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
10	27h	Elevated Breathing	<1	41%

Major Findings: Sustained elevated breathing (avg 25, peak 41)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 25	9 AM to 3 PM	11h	3	Elevated Breathing	25	16	32	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 28	12 AM to 1 AM	16h	7	Elevated Breathing	25	15	33	Check SpO2 and lung sounds; assess for fluid overload or early infection

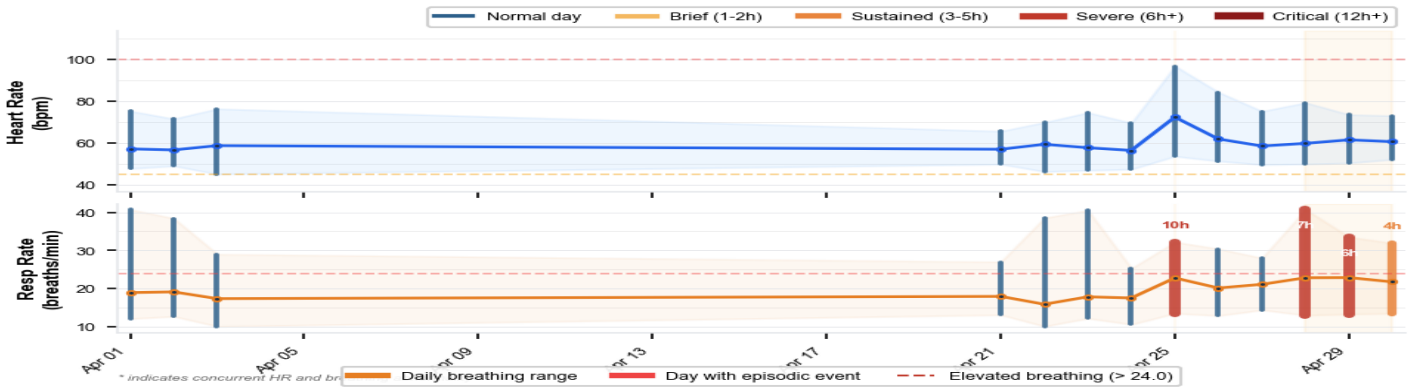
Low data coverage (41%); interpret with caution.

Suggested Clinical Review Actions

- Elevated Breathing (avg 25 brpm, peak 33 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 2 unstable phases with 17 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

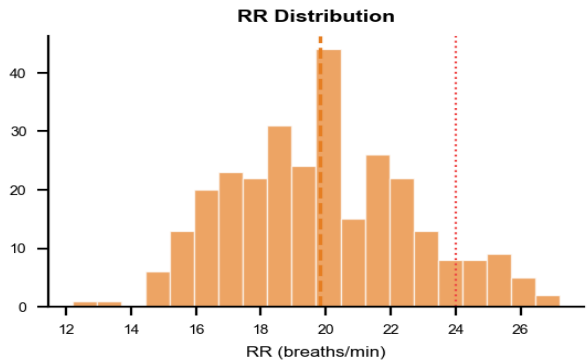
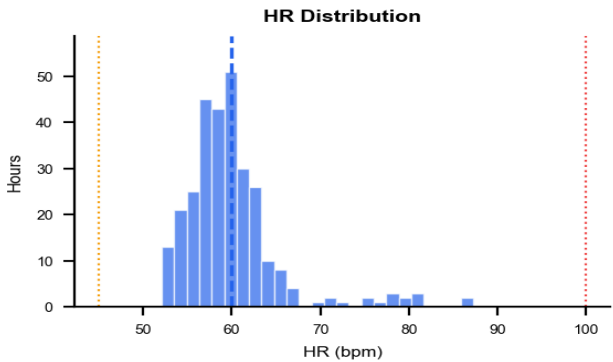
Clinical Pattern Observations:

- 1. **Sustained finding:** 6h continuous Elevated Breathing on Apr 25.
- 2. **Clustered pattern:** Episodic events on 4 of 30 days (13%).

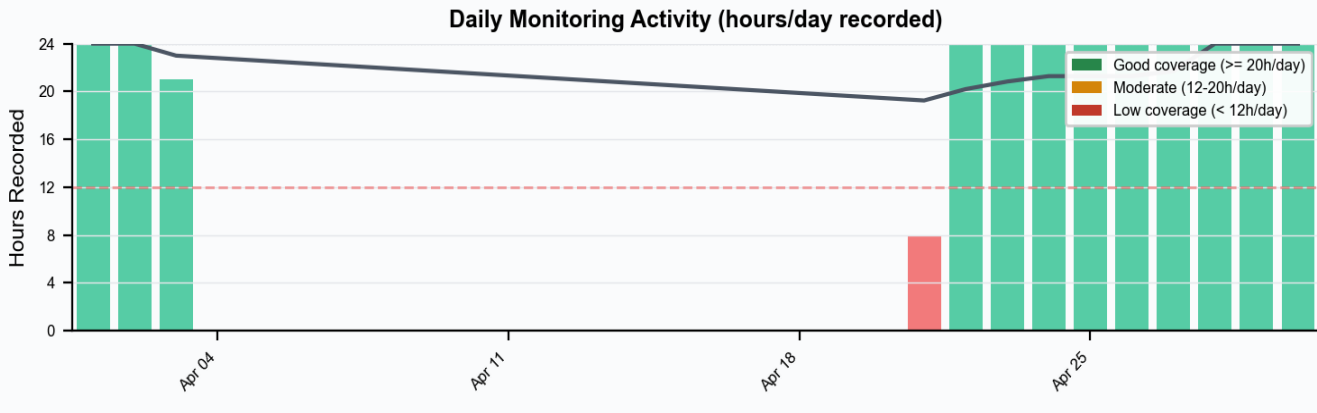


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.